

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

December 10, 2020

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F) MS
Chief of Staff (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C (b)(6),(b)(7)(C),(b)(7)(F)
Acting Chief, Investigations Unit
ICE Health Service Corps

FROM: CDR (b)(6),(b)(7)(C),(b)(7)(F) RN (b)(6),(b)(7)(C),(b)(7)(F)
Investigator
ICE Health Service Corps

SUBJECT: Mortality Review – Report of Findings
Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)
(a.k.a Onoval PEREZ)

Executive Summary:

On July 13, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Onoval PEREZ-Montufa. During his detention period, ICE housed Mr. PEREZ, a 51-year-old Mexican male, at Glades County Detention Center (GCDC) in Moore Haven, Florida (FL) from June 24 – July 12, 2020, the date of his death. An autopsy ruled Mr. PEREZ's death related to COVID-19 and pneumonia.

On June 1, 2020, and prior to Mr. PEREZ ICE custody, Enforcement and Removal Operations (ERO) Boston encountered Mr. PEREZ at Devens Federal Medical Center (FMC-D) in Ayer,

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)
Page 2 of 9

Massachusetts. On June 8, 2020, ERO Boston served Mr. PEREZ a Final Administrative Removal Order, form I-851, charging removability as an alien convicted of an aggravated felony. On June 15, 2020, ERO transferred Mr. PEREZ from FMC-D to Krome Service Processing Center (KSPC) in Miami, FL, for housing and again on June 24, 2020, to GCDC, where he remained for the duration of his ICE custody.

During Mr. PEREZ's custody at KSPC and GCDC, he reported a history of diabetes mellitus [(DM); a chronic condition characterized by high blood sugar, because the body does not produce enough insulin to meet its needs], hypertension [(HTN); a chronic condition characterized by elevated blood pressure], hyperlipidemia (high levels of fat in the blood), and neuropathy (numbness), managed with medications. Mr. PEREZ reported having a coronary artery bypass graft [(CABG); surgical procedure to treat a blockage or narrowing of one or more coronary arteries] in 2017 and received treatment for active tuberculosis in 2015. On June 15, 2020, a KSPC registered nurse (RN) documented a negative coronavirus (COVID-19) symptom screening, completed Mr. PEREZ's medical intake screening, and COVID-19 nasal swab testing [negative]. Upon completion of Mr. PEREZ's intake screening, KSPC medical staff cohorted Mr. PEREZ for 14 days due to KSPC's COVID-19 protocols. On June 24, 2020, in preparation for his transfer to GCDC, an RN completed Mr. PEREZ's COVID-19 symptom screening, transfer summary, which included his negative COVID-19 test results, and documented a temperature (T) of 98.0 degrees (°) Fahrenheit (F) (normal 97.8 – 99.1°F).

On June 25, 2020, Mr. PEREZ arrived at GCDC and a licensed practical nurse (LPN) completed his influenza test [negative], COVID-19 symptom screening and documented a T of 98.0°F. The LPN cleared Mr. PEREZ for housing into general population.

As part of GCDC's COVID-19 protocol, medical staff assess detainees' temperature two times daily. On June 29, 2020, the medical doctor (MD) evaluated Mr. PEREZ for an elevated T of 101.5°F, ordered a chest x-ray, completed a COVID-19 nasal swab test, and admitted Mr. PEREZ into a respiratory isolation room, located in the medical housing unit (MHU). On June 30, 2020, Mr. PEREZ complained of body aches but denied any other symptoms. An LPN evaluated Mr. PEREZ in the MHU, documented an elevated T of 101.8°F, oxygen saturation [(SpO2); fraction of oxygen saturated hemoglobin in the blood] of 94 percent [(%); normal 95-100%], notified the on-call MD and received orders for ascorbic acid (vitamin supplement), zinc (mineral supplement), pyridoxine (vitamin supplement), and acetaminophen (pain reliever and fever reducer).

On July 1, 2020, during MHU rounds, an LPN and an MD observed Mr. PEREZ having generalized weakness, fatigue, difficulty moving from lying to sitting position, shortness of breath, T of 99.0°F, and SpO2 of 90%. The MD gave a verbal order to apply supplemental oxygen at two liters per minute (L/m) via nasal cannula (NC), and transfer Mr. PEREZ via custody transport to Lakeside Medical Center (LMC) in Belle Glade, FL, for further evaluation. Upon arrival to LMC, Mr. PEREZ denied any pain. His vital signs (VS) were normal except for

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)

Page 3 of 9

an elevated blood pressure (BP) of 173/62 millimeters of mercury [(mmHg); normal 90/60 – 120/80 mmHg], T of 100.3°F, and an SpO2 of 90% on room air. An LMC MD evaluated Mr. PEREZ and ordered an electrocardiogram [(EKG); measures the electrical activity of the heart], chest x-ray (CXR), laboratory studies, including COVID-19 test, and admitted Mr. PEREZ to the telemetry unit (hospital unit that closely monitors a patients' vital signs) with a diagnosis of pneumonia and acute kidney failure.

Between July 2 – 8, 2020, Mr. PEREZ's medical condition declined. He remained febrile (showing symptoms of a fever), dyspneic (difficulty breathing), developed pulmonary vascular congestion, and poor renal function. On July 2, 2020, an MD evaluated Mr. PEREZ, noted his febrile status, documented positive COVID-19 results, and elevated inflammatory markers (used to monitor inflammatory conditions). An infectious disease specialist, pulmonologist, nephrologist, and cardiologist evaluated Mr. PEREZ, and he received acetaminophen, intravenous (IV) and oral antibiotics, and convalescent plasma transfusion.

On July 7, 2020, Mr. PEREZ became confused, agitated, and refused to use a non-rebreather mask for supplemental oxygen, which resulted in decreased SpO2 levels, and subsequent transfer into the intensive care unit (ICU), where he received an IV diuretic (medication used to remove excess fluid as a result of pulmonary vascular congestion), twice daily. On July 8, 2020, an MD intubated (insertion of a tube in the trachea to maintain an open airway) Mr. PEREZ and placed him on ventilatory support due to acute respiratory distress. On July 12, 2020, LMC medical staff announced a medical emergency. The medical responders palpated a pulse, and an ICU nurse administered respirations with a bag-valve mask without the endotracheal tube in place. An MD arrived, attempted to intubate Mr. PEREZ, but Mr. PEREZ became pulseless as advanced life saving measures continued. Mr. PEREZ never regained consciousness, and after approximately twenty minutes of life saving measures, the MD pronounced Mr. PEREZ dead at 4:24 p.m.

Mortality Finding:

As a result of Mr. PEREZ's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) determined that Mr. PEREZ's medical care at KSPC and GCDC was not provided within the safe limits of practice. Specifically, the MRC determined KSPC and GCDC's care, or lack thereof, could pose a significant future risk. Overall findings identified were: 1) KSPC's lack of documentation for blood pressure monitoring, medication refusal counseling, and other medical interventions; 2) GCDC custody staff transferred Mr.

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)

Page 4 of 9

PEREZ to the hospital with a low SpO2 level and without supplemental oxygen; and 3) GCDC did not timely refer Mr. PEREZ for higher-level care, despite his multiple co-morbidities and COVID-19 related symptoms.

The MRC convened on November 9, 2020, and consisted of the following members: Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) Doctor of Osteopathic Medicine, South Texas Detention Center's Clinical Director, physician reviewer; CDR (b)(6),(b)(7)(C),(b)(7)(F) Physician Assistant – certified, advanced practice provider (APP) reviewer; Captain (b)(6),(b)(7)(C),(b)(7)(F) Central Regional Nurse Manager, nurse reviewer; Lieutenant Commander LCDR (b)(6),(b)(7)(C),(b)(7)(F) Health Care Compliance Consultant, risk management reviewer.

The following summary of events was based on: 1) Mr. PEREZ's KSPC, GCDC, LMC, Kendall Regional Hospital (KRH) medical records, and emergency medical services (EMS) reports; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. PEREZ's GCDC detention file; 5) video footage from June 30 – July 1, 2020; 6) a remote review and staff interviews conducted by fact-finder CDR (b)(6),(b)(7)(C),(b)(7)(F) on September 8 – 10, 2020; and 7) applicable GCDC and IHSC policies and procedures, and ICE Detention Standards.

Summary of Events

On June 15, 2020, a KSPC RN documented a negative COVID-19 symptom screening, completed Mr. PEREZ's medical intake screening, COVID-19 nasal swab testing (negative), and noted normal VS except for an elevated BP of 166/74 mmHg and an elevated blood sugar (BS) of 201 milligrams/deciliter [(mg/dl); normal range 70 – 100 mg/dl]. Mr. PEREZ reported a history of DM, HTN, hyperlipidemia, and neuropathy managed with the following medications: aspirin 81 milligrams (mg), one tablet by mouth, daily; minoxidil (anti-hypertensive) 2.5 mg, two tablets by mouth, daily; labetalol (anti-hypertensive) 300 mg, two tablets by mouth, three times daily; ergocalciferol (vitamin supplement) 50,000 unit, one capsule two times daily; senna (treats constipation) 8.6 mg, two tablets by mouth, two times daily, as needed (prn); amitriptyline (antidepressant) 50 mg, one tablet by mouth, at bedtime; clopidogrel bisulfate (prevents platelets from forming harmful clots) 75 mg, one tablet by mouth, daily; furosemide (treats fluid retention) 40 mg, one tablet by mouth, daily; amlodipine (anti-hypertensive) 10 mg, one tablet by mouth, daily; atorvastatin (treats high cholesterol and triglyceride levels) 80 mg, one tablet by mouth, at bedtime; pregabalin (treats nerve and muscle pain) 75 mg, insulin neutral protamine hagedorn [(NPH); used for diabetes management] 40 units, subcutaneous injection, two times daily; acetaminophen (pain reliever) 650 mg prn, nitroglycerin (vasodilator used to treat chest pain) 0.4 mg, tablet sublingual, prn, and glucose 4 grams tablet by mouth, prn for hypoglycemia (low blood sugar). Mr. PEREZ also reported having a CABG in 2017 and received treatment in 2015 for active tuberculosis.

On June 16, 2020, a KSPC APP completed Mr. PEREZ's physical exam, reviewed his intake

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)

Page 5 of 9

screening, acknowledged his medical history, and documented additional history findings of left retinal detachment, left great and fourth toe amputations (2017), and continued Mr. PEREZ's medications for DM, HTN, hyperlipidemia, and neuropathy. The APP documented Mr. PEREZ's normal VS, except for an elevated BP of 189/92 mmHg. The APP administered Mr. PEREZ's anti-hypertensive medication, re-checked his BP, which decreased to 173/70 mmHg, ordered a low fat, low sodium/cholesterol diet, laboratory studies, and an EKG.

On June 20, 2020, an APP evaluated Mr. PEREZ for intermittent shortness of breath, moderate chest pain, pitting edema, and dry mouth. His EKG showed normal sinus rhythm, and his VS were: BP of 147/69 mmHg, pulse [(P); normal range 80 – 100 beats per minute (bpm)] of 57 bpm, and a BS of 225 mg/dl. The APP ordered supplemental oxygen at 2L/m via NC, nitroglycerin 0.4 mg, and activated EMS to transport Mr. PEREZ to KRH, in Miami, FL. Upon arrival to KRH, an MD evaluated and admitted Mr. PEREZ for further management. Mr. PEREZ received a computed tomography [(CT); a radiological study that provides a detailed internal view of the body] scan, which revealed pulmonary edema, small bilateral pleural effusions (buildup of fluid between the tissues that line the lungs and the chest), and bilateral lower extremity peripheral vascular disease and a CXR showed evidence of congestive heart failure. A cardiologist evaluated Mr. PEREZ, determined his cardiac condition as stable with sinus bradycardia, first degree atrioventricular block (a type of heart block that occurs when the electrical signal traveling from the upper chamber to the lower chamber is impaired), and elevated troponin levels consistent with chronic myocardial injury pattern.

On June 22, 2020, Mr. PEREZ received a stress test (shows how well the heart handles physical activity), however, the results were pending at the time of discharge and no further work-up was completed due to Mr. PEREZ's refusal for further treatment. A KRH MD discharged Mr. PEREZ with a diagnosis of typical chest pain, elevated BP of 195/67 mmHg and T of 99.1°F. Later that night, Mr. PEREZ returned to KSPC and an RN evaluated him and noted an elevated BP of 201/76 mmHg. Mr. PEREZ denied having any symptoms, reported completing two stress tests earlier that day, and not taking any medications for the past two days. The RN encouraged him to increase fluid intake and performed a manual BP check one hour later, which showed a decreased BP of 145/80 mmHg. The RN notified the on-call APP and received a verbal order to administer labetalol 600 mg, and to continue all other previous medications. An APP did not complete a post hospital evaluation on Mr. PEREZ prior to his transfer to GCDC.

On June 24, 2020, ERO Miami transferred Mr. PEREZ to GCDC. KSPC completed his COVID-19 symptoms screening, prior to Mr. PEREZ departure and documented a normal T of 98.0°F.

On June 25, 2020, a GCDC LPN completed Mr. PEREZ's influenza, COVID-19 symptom screening, intake screening, documented a T of 98.0°F, and elevated BP of 170/66 mmHg. The LPN reviewed Mr. PEREZ's transfer summary, acknowledged his medical history, notified the on-call MD, and received a verbal order to continue his DM, HTN, and hyperlipidemia medications.

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)

Page 6 of 9

On June 29, 2020, an MD evaluated Mr. PEREZ for an elevated T of 101.5°F and left ear pain. Mr. PEREZ denied any other symptoms but reported receiving two COVID-19 laboratory tests previously at FMC-D and KSPC. The MD noted an unremarkable physical examination and stable VS, except for an elevated BP of 155/70 mmHg. The MD ordered a CXR, completed a COVID-19 nasal swab test, and placed Mr. PEREZ into a respiratory isolation room in the MHU.

On June 30, 2020, an LPN evaluated Mr. PEREZ in the MHU's respiratory isolation room, documented the following VS: T of 101.8°F, P of 103 bpm, and SpO2 of 94%. Mr. PEREZ complained of body aches but denied any other symptoms. The LPN notified the MD and received a telephone order to start ascorbic acid, zinc, pyridoxine, and acetaminophen.

On July 1, 2020, an LPN notified the oncoming LPN of Mr. PEREZ's unimproved condition. An LPN and MD completed rounds in the MHU, observed Mr. PEREZ having generalized weakness, fatigue, mobility difficulty, shortness of breath, T of 99.0°F, and SpO2 of 90%. The MD gave a verbal order to administer supplemental oxygen at 2 L/min via NC and to transfer Mr. PEREZ via custody transport to LMC emergency department (ED) for evaluation.

On July 1, 2020 at approximately 11:14 a.m., Mr. PEREZ arrived at LMC ED. An MD evaluated him for shortness of breath, dry cough, fever, and weakness. Mr. PEREZ's VS were normal except for an elevated BP of 173/62 mmHg, T of 100.2°F, SpO2 of 90% on room air. While in the ED, Mr. PEREZ received antibiotics, acetaminophen, anticoagulant medication and oxygen at 2 L/min via a non-rebreather mask. The MD ordered an EKG (normal), CXR (which showed possible small right pleural effusion, diffuse bilateral interstitial opacity), laboratory studies, which included a COVID-19 test, and admitted Mr. PEREZ to the telemetry unit with a diagnosis of pneumonia and acute kidney failure.

On July 2, 2020, the MD evaluated Mr. PEREZ, noted his febrile status, documented positive COVID-19 results, and elevated inflammatory markers. The MD consulted a pulmonologist, nephrologist, cardiologist, and infectious disease (ID) specialist and considered initiating COVID-19 convalescent plasma (CCP) transfusion and remdesivir (anti-viral medication). After further discussion with the other consultants, the MD's decided to hold Mr. PEREZ's anti-hypertensive, diabetes, and hyperlipidemia medications.

On July 3, 2020, a nephrologist evaluated Mr. PEREZ, ordered a renal ultrasound [(US); diagnostic imaging to view the kidneys], and urinalysis. The renal US showed normal results, but the urinalysis, showed moderate blood and significant protein. The nephrologist suspected possible diabetic nephropathy (progressive kidney disease). The ID specialist also evaluated Mr. PEREZ, but did not recommend initiating remdesivir, due to Mr. PEREZ's poor renal function.

On July 4 -5, 2020, Mr. PEREZ became non-compliant with his supplemental oxygen therapy, removed his non-rebreather mask that resulted in decreased SpO2 levels, an increased P, and

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)

Page 7 of 9

unresponsiveness. The rapid response team responded, reapplied the non-rebreather mask, which improved Mr. PEREZ's condition, and responsiveness. After Mr. PEREZ experienced worsening dyspnea, an MD ordered a CXR and an echocardiogram (an ultrasound of the heart). The CXR results showed an enlarged heart, pulmonary vascular congestion, and diffuse interstitial edema. The echocardiogram showed mitral and tricuspid regurgitation, and diastolic dysfunction. A cardiologist evaluated Mr. PEREZ and recommended continuation of IV diuretics.

On July 6, 2020, an MD evaluated Mr. PEREZ, noted CXR results consistent with congestive heart failure, and placed Mr. PEREZ on a bi-level positive airway pressure [(BiPAP); used to increase intrapulmonary pressure to reduce pulmonary vascular congestion] machine.

On July 7, 2020, Mr. PEREZ became confused, agitated, and refused to wear the non-rebreather mask, which resulted in decreased SpO2 levels, requiring his subsequent transfer into the ICU, IV diuretic administration twice daily, and a CCP transfusion.

On July 8, 2020, an MD intubated Mr. PEREZ and placed him on ventilatory support, due to acute respiratory distress.

On July 12, 2020 at approximately 3:50 p.m., LMC's medical staff announced a medical emergency for Mr. PEREZ. The responders palpated a pulse, and an ICU nurse administered respirations with a bag-valve mask without the endotracheal tube in place. The ED MD reported to Mr. PEREZ's room, attempted to intubate him, but Mr. PEREZ became pulseless as advanced life saving measures continued. Mr. PEREZ never regained consciousness, and after approximately twenty minutes of life saving measures, the MD pronounced him deceased at 4:24 p.m.

The final autopsy report revealed Mr. PEREZ's cause of death was related to COVID-19 pneumonia and contributory causes of congestive heart failure, atherosclerotic (a condition where a build-up of cholesterol plaque in the artery walls causes obstruction of blood flow) and hypertensive cardiovascular disease. The coroner determined the manner of death as natural.

Detention standards used for this review: ICE National Detention Standards (NDS), 2019, and the National Commission on Correctional Health Care Standards (NCCHC), 2018.

The following is a summary of health care delivery/program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) GCDC medical staff assess detainees' temperature two times daily; and 2) after Mr. PEREZ's death, GCDC medical staff initiated COVID-19 nasal swab test on all new arrivals.

Mortality Review – Onoval PEREZ-Montufa, (b)(6),(b)(7)(C)
Page 8 of 9

Weaknesses

1. Timely referral

- On June 22, 2020, Mr. PEREZ returned to KSPC from KRH; however, an APP failed to evaluate him prior to his transfer to GCDC on June 24, 2020.
- On June 29 – 30, 2020, Mr. PEREZ presented with an elevated T, P and body aches; however, GCDC did not transfer Mr. PEREZ to the local ED until July 1, 2020, despite his multiple co-morbidities.

Applicable standards of care for this finding:

- ICE National Detention Standard (NDS) 2019: 4.3, Medical Care; section II.A. *General*.
- NCCHC 2018: J-D-08, *Hospital and Specialty Care*.
- NCCHC 2018: J-E-09, *Continuity, Coordination, and Quality of Care During Incarceration*.
- Armor Correctional Health Services, Inc., Glades County Detention Center (GCDC) *Continuity, Coordination, and Quality of Care During Incarceration*, J-E-09, February 2020.

2. Patient safety.

- On July 1, 2020, an LPN documented Mr. PEREZ's SpO2 level of 90%, dyspnea and fatigue. GCDC custody staff transported him to LMC, which is approximately 65 miles away, without supplemental oxygen.

Applicable standards of care for this finding:

- NCCHC 2018: J-B-08, *Patient Safety*.
- Armor Correctional Health Services, Inc., GCDC, *Patient and Staff Safety*, J-B-08, October 2019.

3. Documentation.

- On June 18, 2020, while housed at KSPC, Mr. PEREZ complained of lightheadedness, but KSPC's staff did not document orthostatic checks.
- Between June 19 – 24, 2020, while at KSPC, Mr. PEREZ refused his medication and signed seven refusal forms. The nursing staff referred him to a provider for medication counseling; however, there is no documentary evidence to support Mr. PEREZ received counseling for his medication refusals.
- While at KSPC, Mr. PEREZ's BP consistently remained elevated; however, there was no documentary evidence to support he received BP checks or monitoring.

Mortality Review – Onoval PEREZ-Montufo, (b)(6),(b)(7)(C)

Page 9 of 9

- On June 29, 2020, a GCDC LPN documented Mr. PEREZ's elevated temperature, but did not document Mr. PEREZ's pulse oximetry reading.

Applicable standards of care for this finding:

- IHSC Directive, 02-08, *Refusal of Treatment*, March 2016.
- IHSC Operations Memorandum (OM), 16-022, *IHSC Health Records Management*.
- IHSC OM, 16-027, *Clinical Nursing Guidelines (Interim RN Guidelines)*.
- NCHC 2018: J-A-08, *Health Records*.
- Armor Correctional Health Services, Inc., GCDC, *Health Records*, J-A-08, February 2020.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, the GCDC administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the CAP is implemented and sustained.

End of report.